

Institute of Actuaries of India

Subject ST1 – Health and Care Insurance

Specialist Technical

May 2012 Examinations

INDICATIVE SOLUTIONS

Introduction

The indicative solution has been written by the Examiners with the aim of helping candidates. The solutions given are only indicative. It is realized that there could be other points as valid answers and examiner have given credit for any alternative approach or interpretation which they consider to be reasonable.

Solution 1 :**a) Possible reasons for increasing healthcare expenditure**

- Free cover and so over utilization – people tend to believe that more care is better and not worried about cost as the Govt. pays for it; No incentive for individuals to manage their health
- Health care providers not incentivized for efficient utilization – so, providers aim at maximizing their revenue in each opportunity as opposed to looking at efficiency
- Lack of focus on preventive care - Increased prevalence of lifestyle diseases and inadequate awareness about their impact; no incentive for health care providers to focus on preventive care
- Medical trend – medical technological advancements making it less painful to undergo certain treatments, new and costly treatments for some diseases now available, and increasing medical service costs – leading to higher utilization/cost
- Health of the population has been deteriorating due to new diseases, life style changes, Civic infrastructure not keeping pace with the increasing population etc.
- Fraud in the management of health and care provision leading to inflated costs

b) Actions that the Government can take to encourage participation in the scheme

- Contribution to the account tax deductible
- Supplemental contribution by the State
- Withdrawals from the account non-taxable
- Reduced general taxes to those who participate
- Mandatory deduction from the salary for employed people
- Offer guaranteed/attractive investment returns for the savings
- Priority to those participate against those who don't in terms of access to certain health care
- Easy and user-friendly administration – hassle-free withdrawal of money from the account, direct payment to the healthcare service providers etc.

c) Gaps in the proposed scheme and how they can be mitigated**Gaps**

- Outpatient and day care treatments not covered => disgruntled people
- May encourage unnecessary hospital stays to meet 24 hrs hospitalization requirement and so will push up the cost of health care – this would depend upon what happens to any unutilized money in the individual accounts
- Not all individuals will be able to decide how much they should save in order to meet the future medical expenses

- Basic healthcare will be kept affordable – the ‘affordability’ is a relative term, the poorest may not be able to afford even what is termed ‘affordable’
- The administrative burden of running the scheme may increase the cost of the provision of care – would mean less ‘value for money’ for those saving into the accounts
- People who pay taxes might feel they are double-paying for the health care benefits and so will be less encouraged to save into the account
- Not all the people (e.g. unemployed, retired, disabled and children) can afford to pay the sufficient amount into the healthcare account
- If the investment return earned on the savings account is poor compared to other investment avenues available, there will likely be more people discontinue making contributions into the scheme and so will be left with inadequate funds when need arises
- The limits defined for each treatment is difficult to decide and will require regular validations due to medical inflation and advancement.

Solutions

- Offer cover free of cost to certain categories of people (e.g. people below poverty line) at least for major illnesses
- Remove the 24 hour hospitalization requirement and rather have a higher co-pay (individuals pay greater proportion of the cost) for outpatient and day care treatments
- Get an insurance company to professionally manage the scheme and so the individual contributions are varied over time taking into account their claim history/risk profile so that there is minimal cross-subsidy and there is adequate fund to provide the health care as they arise
- Encourage judicial spending on health care by offering monetary incentives – for example, double the balance in the account on reaching retirement age to take care of post-retirement medical and long term care needs
- Allow the individuals to choose the type of investment for their contributions based on their risk appetite subject to minimum investment guarantees

d) Factors to consider in deciding upon an investment policy

- Main objective is to maximize investment returns subject to an acceptable level of risk
- Match characteristics of the assets with those of the scheme’s obligations by term, amount, nature and currency
- Nature
 - o Guaranteed in money terms (e.g., fixed package for certain types of treatments negotiated with the providers)- Invest in fixed interest securities
 - o Guaranteed in terms of an index of prices or similar (e.g., claim expenses) - Invest in indexed linked securities.

- o Indemnity based (e.g., outpatient expenses) - Invest in securities that are likely to increase in line with the cost of medical inflation.
- Term - The term of the assets should match with the expected benefits outgo. For example, short term to meet the liquidity requirements. Otherwise, it can result in liquidity crunch on one hand or reinvestment risk on the other.
- Currency- As most of the obligation under the scheme is expected to be in local currency, investment in a different currency will lead to currency risk/ exchange rate changes.
- Need to allow for the frequency of contributions – e.g., monthly, yearly etc.
- Marketability of assets – as long as the fund remains small, claims may fluctuate from one month to the next
- Consider outsourcing the entire investment to professional investment managers to give confidence to the investing public that their money is being taken care of
- Political sensitivities may mean less riskier investments and minimum investment guarantees
- Need to allow for catastrophic events requiring large sums of money to be paid out in a short span of time– e.g. pandemic

[15]

Solution 2 :**a) Reasons why PMI product's claims experience may vary from that assumed in the premium basis**

- Lack of previous experience
- Variability of experience
- Changing mix of business
- Changing risk characteristics
- Anti-selection by policyholders
- Changing attitudes to claiming
- Climate effects
- Catastrophes
- Latent claims
- New types of claim
- Claims inflation
- Legislative changes
- Judicial changes
- Regulatory changes
- Medical advancements not anticipated
- Poor claims management
- Poor expense management
- Poor underwriting
- Fraud

b) A/E ratios

		Incidence rate		2.50%								
		Avg. claim size		27,000								
		Claim cost per life per year		675								
CY		2009	2010	2010	2010	2010	2010	2011	2011	2011	2011	2011
		Q4*	Q1	Q2	Q3	Q4	FY	Q1	Q2	Q3	Q4*	FY
# Lives insured (EOQ)	1	19,120	21,345	38,559	37,272	45,098		50,345	48,974	60,904	73,692	
# Lives (Average)	2		20,233	29,952	37,916	41,185	129,285	47,722	49,660	54,939	67,298	219,618
# of claims (expected)	3=(2)*2.5%		506	749	948	1,030	3,232	1,193	1,241	1,373	1,682	5,490
# of claims (actual)	4		547	925	1,087	1,987	4,546	1,345	1,296	1,234	2,878	6,753
Incidence Rate	5=(4)/(2)		2.7%	3.1%	2.9%	4.8%	3.5%	2.8%	2.6%	2.2%	4.3%	3.1%
Actual claims('000)	6		13,254	21,974	33,673	67,073	135,975	34,253	34,703	40,053	104,198	213,207
Expected claims('000)	7=(3)*27000		13,657	20,218	25,593	27,800	87,268	32,212	33,520	37,084	45,426	148,242
Avg. claim amount	8= (6)/(4)		24,231	23,756	30,978	33,756	29,911	25,467	26,777	32,458	36,208	31,574
A/E Ratio	9=(6)/(7)		97.1%	108.7%	131.6%	241.3%	155.8%	106.3%	103.5%	108.0%	229.4%	143.8%
<u>Assumptions</u>												
The rate of change in no. of lives insured from one quarter to next is the same for years 2009-2011 - to estimate the # Lives EOQ for Q4'09 & Q4'11												
Entries (eg. New policies) and exits (eg. lapse) are uniformly distributed within a quarter												
Seasonality (in Q4) in terms of claim size and incidence are at the same level in 2010 and 2011 - to estimate Q4'11 incidence and claim size												
Total amount claimed is same as the total amount settled												
All claims incurred upto 30.9.2011 has been reported as of 31.12.2011												

[A/E for Y2010– 1.5 marks; for Y2012- 2 marks; Assumptions – 1.5 marks; Max– 5 marks]

c) Setting assumptions to determine expected claim cost per life per annum

- Broad steps involved:
 - Investigate the historical experience and make best estimates of the parameters from that experience
 - Consider what the conditions (including the commercial and economic environment) will be like in the future period for which the assumptions are to be applied
 - Determine what the best estimates of your assumptions will be, given the expected future conditions. The extent to which one would rely on the experience data, and the extent to which you allow for other factors, including judgment, depends on the credibility and the relevance of the data, and how predictable the parameter is
- Investigating the historical experience
 - Claim cost per life per annum = incidence rate x average cost per claim
 - The factors influencing the two components may vary and so they need to be considered separately

- o Use past data subdivided by benefit/procedure class, age, gender, smoker status, occupation, distribution source, territory, region
 - o Claims split by causes so to get a sense of the size of claims by different
 - o Analyze delay in reporting of claims
 - o Analyze the amount claimed vs. amount settled to predict trend of any fraudulent trends
 - o Consider the impact of seasonality in the claim occurrence (e.g. rainy season may show greater incidence of infectious diseases)
 - o Comparing the average cost per claim historically will give an indication of the likely medical inflation in the past
 - o Ensure the data is split into reasonably homogeneous risk profiles subject to credible data being available in each cell
 - o Period of investigation should not be too long to be less relevant
- Adjust for the future – allow for
 - o Medical trends – both level of utilization and the size of claims - new diseases, new methods of treatments etc. – consider past trends, medical resources demand and supply, Price and Earnings inflation etc.
 - o Changes in underwriting, claim management
 - o Allow Changes in risk characteristics
 - o Regulatory changes
 - o Rate guarantees and the practical limitations on increasing premium rates
 - o Impact of reinsurance

d) Calculation of Embedded value

Embedded value can be calculated as the sum of:

- **The shareholder-owned share of net assets, where**
 - o The net assets are defined as the excess of assets held over those required to meet liabilities. These assets may be valued at market value or discounted to reflect “lock-in”, for example if they are required to be retained within the fund to cover solvency capital requirements, and
- **The present value of future shareholder profits arising on existing business**
 - o this would be the present value of future premiums plus investment income less claims less expenses plus the release of solvency reserves.
- Calculations may be done using full data or model points.
- Future cash-flows will be discounted at the risk discount rate, reflecting the shareholders' required rate of return. Experience assumptions may be best estimate, but this depends on the purpose of the embedded value calculation. Tax is allowed for within the calculation as appropriate.

- It is important that the reserves used in the determination of net assets are the same as those used in the determination of the present value of future profits.
- The start point in deciding on a suitable basis will be the basis used for the previous embedded value calculation. Any differences will immediately cause some movement in the embedded value. Thus any such change must be justifiable, especially if the embedded value is being used to report externally on a company's real worth.
- Embedded value basis might be more "best estimate" than is the pricing basis – incidence rate, claim size, medical trend and expenses. However, the two should be looked at side by side. Any differences will immediately lead to embedded value movements on writing new business different from those implied in the pricing basis.
- An embedded value calculation will consider the cash-flows across the portfolio in each time period, rather than the present value of cash-flows for each policy. The focus of an embedded value calculation is shareholder profit. Shareholders are interested in the present value of the actual distributions of profit that will be made to them in the future.
- For an embedded value, expected amounts of the shareholders' transfer each year are calculated, and discount these at an appropriate risk discount rate.
- An embedded value therefore takes full account of the cost of capital in the value of these transfers, because each year's transfer has to be made from profits that arise in excess of the supervisory reserves held.

[24]

Solution 3 :

a) Main product specific risks

- Morbidity Risk including Anti Selection: Claims higher than expected due to model, parameter or random fluctuation risk.
- Expense Risk: Expenses higher than expected. (sales, underwriting, renewal or claims expenses)
- Selective Withdrawal: Healthy lives withdrawing and poor lives continuing.
- Moral hazard Risk: Hospitals and/or patients may not be willing to discharge early as they know that for each day of stay insurance company will pay the benefits
- Reputational Risk: If the benefits are fixed per day then policyholders may not see as a value as actual stay cost can be much higher than the fixed benefits payable by insurer.
- Counterparty risk: Reinsurers, service providers, brokers etc.
- Catastrophe Risk: Epidemic Outbreak

- New Diseases and medical advancement: New diseases can increase the incidence of hospitalization whereas medical advancement is likely to reduce the length of stay per hospitalization.

b) Pricing implications of waiting period change

- Premium charge may be lower compared to earlier design as minimum stay requirement in hospitalization has changed from 24 hours to 48 hours
- Incidence rate is likely to reduce as minimum stay requirement is increased to 48 days
- Average Length of Stay- It is expected that persons staying for more than 48 hours are suffering more or getting treated for major conditions. Hence, the average length of stay is likely to increase in this case
- Lower no. of claims leading to lower claims handling expenses
- Other things being equal, if the volume reduces due to increased waiting period, this will lead to increase per policy expenses and so higher premium
- If the similar type of plans is offered by competitor at cheaper price then it may lead to high lapses of this policy that needs to be allowed for in the pricing

c) Pricing implications of introducing 1 day deductible

- Claims incidence rates will not change as all claims that are currently reported will continue to be reported
- Average Length of Stay- The average length of stay will not change. However, given the one day deductible, for pricing purposes the average length of stay will be reduced by 1 day across. This will bring down the cost of claim and hence, the premiums are expected to be lower
- This product design arguable requires greater claims administration. Hence, increasing the claims management cost.
- The expected sales volumes gets affected depending upon the competitive situation of the new product thereby leading to change in the expected per policy expenses.

d) Factors to consider for change in product design

- Profitability of the product: Does it meet the shareholders profit criteria?
- Marketability: How the product compares with other products in the market. Which channel can it be sold through etc.
- Need for competitive rate: Is the target market for which the product is to be launched price-sensitive?
- Financial requirements: The capital requirements (reserving and solvency margins) of the product

- The risks associated with the product and the risk appetite of the insurer. This will influence the product design and need to reinsure.
- The onerous of any guarantees and options embedded into the product. The guarantees carry a cost and also increase the reserving requirements. Can these be passed on to the customers?
- The sensitivity of profit to variations in future experience: Is profit very sensitive to withdrawals? If so can the product design be altered to account for it?
- The extent of cross-subsidies possible between different groups of policyholders. Also, can Expense overruns be cross subsidized with investment income?
- Administration and IT systems: The new product should have administrative requirements similar to that of other existing products unless warranted by significant volumes.
- Consistency with other products: Must not lead to lapse and re-entry.
- Regulatory constraints and opportunities: Must meet regulatory requirements

[12]

Solution 4 :

a) Reasons for difference in premium

- Difference in reviewability period - Ability to review premium/terms every year under the non-life contract would mean lower premium, other things being equal
- Difference in underwriting - This leads to difference in underwriting expense, SA related expenses and overall claims experience.
- Difference in the operating Model: Whether policies are serviced through TPA or in-house leading to difference in expenses.
- Difference in portfolio size: This affects the per policy expenses.
- Difference in reserving requirements: Life and Non-life companies may have different reserving requirements. e.g., Life health contracts have more guarantees and are long term. Hence, reserving requirements may be more prudent leading to higher cost of capital.
- Difference in profitability criteria for the Companies leading to difference in premium charged
- Level of Cross-subsidy across lines of business may differ
- Different reinsurance arrangement leading to difference in ultimate cost to the company
- Different network of hospitals and so differing controls over claims.

b) Factors to consider deciding where to buy from

- Service levels
 - o Ease of buying/paying premiums
 - o cash-less hospitalization facility
 - o Network of hospitals
 - o Claim process turn around / easiness
 - o Level of repudiation in claim
- Certainty of premium over the next 3 years vs. possible change every year
- Front-end (at the time of application) and so more clear as to what is covered and what is not Vs. back-end (at the time of claim) underwriting
- Reviewability terms and conditions – what can change? How?
 - o Only premium? Or, coverage can change as well?
 - o Review based on individual experience or portfolio experience

[6]

Solution 5 :

a) Reserving for the option

- Assumptions required
 - The expected number of the policyholders exercising the option
 - The expected claim experience of the individuals taking up the option
 - The expected expenses of administering the option
- Bear in mind the exercise of the option is likely to be selective - individuals taking up the option are more likely to claim than those who are not
- The lower the number of individuals exercising the option the higher the expected claim experience of those exercising – these two assumptions should therefore be consistent
- Sensitivity test these assumptions and see the impact on resulting reserves; experience with regard to option take up and the resulting morbidity experience will need to be continually monitored and the reserving adjusted
- Allow for the increased claim payout (both the incidence will likely be higher and the amount of payout higher), associated expenses and increased premium in the reserve cash flow model
- Consider the regulatory requirements with regard to the options reserving

b) Risks and mitigation**Risks to the insurer**

- Difficult for the air-hostesses to explain the complex benefit definitions – resulting in poor sales or mis-selling
- The air-passengers are likely to be of higher socio-economic strata and so well aware of the state of their health – risk of anti-selection, especially in a simplified underwriting scenario
- The passengers may not have all the information ready (eg. previous insurance details) and so the proposal form may be incomplete requiring subsequent follow-ups – this may lead to loss of business
- There may not enough time, especially the short-duration flights for the passenger to fill in the proposal form completely – the air-hostesses may fill in the form instead and so may lead to non-disclosure and issues at the claim stage
- Regulator may disallow such complex products to be sold via this channel or impose restrictions on the level of cover – impact on sales

Risk mitigation

- Air-hostesses need to go through adequate training
- Ensure appropriate underwriting
- Have dedicated air-hostesses who are well trained and have adequate time (say, one per flight) as opposed to allowing everyone
- Supply the proposal form as part of the ticketing process and so the passengers will have time to go through the questions and be ready with the answers
- Make it a simplified issue (limited cover with minimal underwriting)
- Claw back of commission (recovery of part of the commission in the event of policy lapsing)
- Monitor the experience by air-hostess, gender, size of cover, flight route etc.
- The air hostesses may be used to market the product and generate the lead. The insurance company staff takes it from there. This may be more acceptable to the regulator.

c) Underwriting guidelines**Initial underwriting**

- The information that need to be sought at the time of application particularly highlighting the difference from the life insurance application
- Description with examples of health insurance terminology such as pre-existing conditions and their significance

- Description of the differences between front-end [fully underwriting at the outset and making it clear what is not covered] and back-end underwriting [limited underwriting at the outset with moratorium for pre-existing conditions]
- Specific health questionnaires that need to be triggered based on any adverse noting in the standard application form
- Clear outline of the product terms and conditions with examples
- Medical/financial underwriting requirements based on the customer profile (age, gender, sum assured etc.)
- Underwriting rules and authorities (including the reinsurance referral requirements)
- Information and documents required for a claim processing
- Importance of dealing with claims sensitively
- The process of claim admission and investigation (including referral to reinsurer)

d) Adding HIV/AIDS to the list of conditions

- It will be seen as serving a social cause and so would likely be welcome by the regulator and enhance the company's brand
- It will likely increase the sales volume if the additional premium for covering the HIV/AIDS is small and/or partly subsidized by the Government
- Provision of payout in the form of annuity as opposed to a lump sum will minimize any potential anti-selection that may otherwise result
- It may be difficult to define HIV/AIDS and there will be need to vary the definition with changing medical advancements
- Underwriting will get expensive as we need to test the applicants for HIV/AIDS at the outset
- If competitors are already providing this benefit, then it would make sense to follow the competition to avoid being singled out
- Those who feel not exposed to the risk of HIV/AIDS may feel the addition of benefit is not a good value for money (depending upon how much extra they would need to pay)
- So, it may be offered as a rider and so those who do not feel the need may not buy it
- There may be no and limited data to estimate the incidence rate for the condition and so some conservatism will be built in the price making the product a bit expensive => can lead to reduction in sales
- Estimating the length of the payout will be extremely difficult as the life expectancy may be significantly improved with medical advancements and so element of conservatism in pricing and reserving will make the product a bit expensive
- People would typically like to have the lump sum payout. That is how the CI products become popular. Also, if the annuity payouts are taxed then the annuity mode of payout will be less appealing

- It may be advisable to limit the cover only to HIV/AIDS caused by inadvertent blood transfusion. But, it may be difficult to lead to challenges at the claim stage when it comes to proving the exact cause of the disease.
- Significant medical advances making the living with HIV/AIDS not so onerous or illness becomes curable then there may be significant anti-selection (voluntarily contracting the disease to avail the benefits)
- Making the annuity payable until death or complete cure, whichever occurs earlier may be a better design to avoid people getting paid even if they are no longer suffering from the disease.
- Reviewability of terms is important given the uncertainty surrounding the incidence and longevity of those living with HIV/AIDS.

[20]

Solution 6 :

a) LTCI rider fit into other products

i) Critical Illness Insurance

- LTCI has a good overlap with CI insurance because many critical illnesses in old age would generate the need for Long term care.
- Common causes of long-term care (eg Alzheimer's disease, stroke, Parkinson's disease, rheumatoid arthritis, pre-senile dementia and blindness) are already covered by the main CI policy
- As there is significant overlap, LTCI rider can be added to the CI policy by charging only a small amount of extra premium. This will be seen as a significant benefit to policyholder by paying a little extra cost.
- Desirable Feature- This is normally structured so that the definition of total and permanent disability changes from occupation-related (or activities of daily work) to the loss of independent existence (i.e. failure of ADLs) at age 60.

ii) Income Protection Insurance

- Long term care can be added to an IP Policy, so that the cover continues beyond normal retirement age.
- However, there is no link between the levels of income protection and the long term care benefits required. The level of income protection benefit depends on earnings, but the level of long-term care benefit depends on the difference between the cost of care and the amount the insured can afford to pay towards care costs.
- Hence, the policy with a rider would resemble two policies packaged together rather than an integrated product. There would be some saving in

expenses, but few other savings. The combined policy would not be much cheaper than buying two separate policies.

- Desirable Feature- At the end of the IP term, the definition of disability would normally switch from being occupation based to one that is activity based.

iii) Private Medical Insurance

- LTCI is a natural rider to the PMI policy as some consumers believe that the benefits provided under LTCI should anyway be provided under PMI Policy.
- PMI pays for acute conditions and LTCI rider pays for the care needed as a result of chronic conditions.
- However, PMI are short term contracts whereas LTCI are expected to be long term contracts.
- Adding LTCI as rider to PMI creates expectation that it will provide for complete care (indemnity) rather than providing fixed cash benefits.
- Desirable Feature - For the rider premium to be not significantly higher than the base PMI cover premium, The LTCI benefit may be restricted, compared to the usual benefits offered i.e. only provide a pre-specified number of hours of home nursing, dependent on the level of disability.

iv) Hospital Cash Insurance

- Again, the LTCI is a natural rider to the Hospital Cash policy as some consumers believe that the benefits provided under LTCI should anyway be provided under PMI Policy.
- While hospital cash plans provide fixed benefit for per day of hospitalization for acute conditions, it can cover LTCI to provide cash benefits to individuals in need of long term care based on certain benefit triggers.
- However, the Hospital cash plans are generally cheaper plans whereas LTCI plans can be expensive. The policyholders buying a hospital cash plan may not be willing to include the LTCI rider.
- Also, Hospital cash plans are short term contracts while LTCI are usually long term contracts.
- Desirable Feature - It will be important to keep the rider premium low to as to incentivize policyholders to take up the LTCI rider. This can be done by restricting the benefits and providing cash benefits rather than provision for care.

b) Typical means of reducing LTCI premium and risks associated**Means of reducing LTCI premium**

- Offering cash benefits as against provision for care
- Separating personal care into nursing care and other forms of personal care and then restricting the benefits covered under nursing care. As nursing care is expensive, restricting it is likely to reduce the premium.
- Providing intermediate care that focuses on recuperative services following an acute event in order to reduce avoidable hospital admission and minimize future dependence on long term care.
- Encouraging informal care by providing an incentive by way of cash payments
- Altering the definition for benefit triggers: Instead of basing it on the level of disability, an additional criterion may be added for benefit trigger. e.g., In addition to satisfying a disability criterion, the insured must also need care during the night or care that can only be provided in the nursing home. The more stringent the definition, the less likely is the event insured and in general the lower the premium will be.
- Adding LTCI as rider to other policies rather than standalone policies. This reduces the administration cost hence, reducing the premium.
- Increasing the length of the deferred period. This reduces the chances of claim payouts thereby, reducing premium.
- Reducing the rate of benefit escalation especially when the policy is in the claim.
- Having an monetary cap on the total benefits payable under the policy
- No death or surrender benefits offered under the policy
- Limit the benefit payment periods
- Restricting the hospitals where the benefits may be provided.
- Altering the ADL definitions and making them stricter
- Not offering any form of guarantees or options to the policyholders. This reduces the reserving requirements thereby reducing the premium.
- Managed care – timely intervention to ensure the type of care provided is appropriate (switch from nursing care to home care etc.)

Associated Risks

- Product seen more restrictive and not meeting the needs and so reduced business volume
- Risk of cost of implementation of these measures outweigh the benefits and so getting the pricing wrong
- Risk of losing control over claims when there are third parties involved (e.g., care counselor, TPA etc.)
- Lack of experience on different design and claim management aspects could lead to potential selection against the insurer

c) How reinsurance can be used to mitigate the risks

- Reinsurers will have access to experience in various markets and so more data and technical help with pricing – to help reduce the mispricing risk
- Reinsurers experience with different product designs (benefit definitions, exclusions etc.) and claim management practices will help manage potential anti-selection risk
- Reinsurance can be used to limit the exposure to risk - Initially, a significant proportion of the risks can be reinsured. As the Company builds experience the retention levels can be increased. This will help the company to avoid any major losses initially while gaining experience and then later on retaining significant proportion of the profit making business.
- Reinsurers can provide technical support on underwriting and claim management
- Reinsurers can help with selection of third parties to be engaged based on their access to knowledge of the quality and effectiveness of those
- Reinsurance can be used to lower the cost of capital thereby helping to reduce premium and thus increase the business volume.

[23]**[Total Marks – 100]**
